

1. Administrative & Study Identification

Full Study Title: A Safety Study of PF-08046044/SGN-35C in Adults With Advanced Cancers (A Phase 1, Open-label Study to Evaluate PF-08046044/SGN-35C in Adults With Advanced Malignancies) **Short Title/Acronym:** SGN35C-001 **Protocol Number:** SGN35C-001 **NCT Number:** NCT06254495 **Sponsor Name:** Seagen, a wholly owned subsidiary of Pfizer **Investigational Product:** PF-08046044 / SGN-35C (CD30-directed antibody-drug conjugate with a topoisomerase 1 inhibitor payload) **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective: To characterize the safety and tolerability of SGN-35C in participants with relapsed/refractory (R/R) lymphomas, determining the type, incidence, and severity of adverse events, and to identify dose-limiting toxicities (DLTs). **Secondary Objectives:** To evaluate pharmacokinetics (PK), preliminary antitumor activity (ORR, CR rate, DOR), and the incidence of antidrug antibodies (ADA).

2.2 Background & Rationale To address the need for novel therapies in relapsed/refractory CD30+ lymphomas (such as cHL, PTCL, sALCL, DLBCL) that have progressed on standard treatments like brentuximab vedotin (BV) or PD-1 inhibitors. CD30 is a validated therapeutic target with limited expression in healthy cells. SGN-35C uses a chimeric IgG1 CD30-directed monoclonal antibody conjugated to a novel camptothecin-derived topoisomerase 1 inhibitor via a protease-cleavable glucuronide linker. This mechanistically distinct payload shows preclinical activity in BV-resistant cell lines and aims to bypass peripheral neuropathy commonly associated with MMAE-based ADCs.

3. Study Design & Methodology

3.1 Design Framework

Study Type: Interventional **Control Type:** Single-arm (First-in-human dose-escalation and dose-expansion) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration

Target \$N\$: Variable (Adaptive Phase 1 enrollment, encompassing Part A dose-escalation, Part B dose-optimization, and Part C dose-expansion cohorts) **Number of Sites:** Global multicenter (including United States, Denmark, France, Italy, Spain, United Kingdom) **Estimated Study Start:** 28-May-2024 **Estimated Primary Completion:** 18-Apr-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants with a histologically confirmed lymphoid neoplasm (cHL, PTCL, sALCL, DLBCL) who have no appropriate standard therapy available. 2. For Parts A & B (dose escalation/optimization): Participants with PTCL and DLBCL must have a detectable CD30 expression level ($\geq 1\%$) in tumor tissue from the most recent biopsy obtained at or after relapse. For Part C (expansion): Eligible irrespective of CD30 expression but must provide tumor tissue for evaluation. 3. Must have received required prior systemic therapies (e.g., at least 3 prior systemic therapies including ASCT or anti-PD-1 agent for R/R cHL; at least 2 prior therapies for R/R DLBCL including CAR T-cell therapy if eligible).

4.2 Exclusion Criteria 1. Previous exposure to any antibody-drug conjugates (ADCs) with a camptothecin-based payload. 2. Active central nervous system (CNS) disease related to the underlying malignancy (participants with treated and clinically stable CNS disease without ongoing steroid treatment are allowed). 3. Active acute or chronic graft-versus-host disease (GVHD) or receiving immunosuppressive therapy for GVHD; previous allogeneic SCT ≤ 100 days prior, or ASCT ≤ 12 weeks prior to the first dose of SGN-35C.

5. Intervention & Treatment Plan

5.1 Investigational Regimen

Dosage/Frequency: Ascending dose levels (e.g., 0.6, 1.2, 2.0, 2.5, 3.2 mg/kg) administered every 3 weeks (Q3W). **Route of Administration:** Intravenous (IV) infusion **Duration of Treatment:** Continued until disease progression per Lugano criteria, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy

Permitted: Standard supportive care for managing disease-related symptoms. **Prohibited:** Other investigational anti-cancer agents, recent live virus vaccines, and concurrent immunosuppressive therapy for GVHD.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Biopsy (for CD30 evaluation), Baseline Imaging
Baseline	Day 1	First IV Infusion (Cycle 1), Baseline PK/PD/ctDNA sampling
Interim	Every 3 Weeks (Q3W)	Safety Labs, Treatment Infusion, PK/ADA collection, Efficacy Assessment (Lugano criteria)
End of Study	+30 to 37 Days	Final Vital Signs, Exit Interview, Safety Follow-up, Survival & Progression Monitoring

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Type, incidence, and severity of adverse events (AEs) and laboratory abnormalities, frequency of dose modifications, and incidence of dose-limiting toxicities (DLTs). **Measurement Method:** Clinical assessments graded per standard toxicity criteria through 30-37 days after the last study treatment.

7.2 Secondary & Exploratory Endpoints * Objective Response Rate (ORR), Complete Response (CR) rate, and Duration of Response (DOR) as assessed by the investigator per Lugano criteria (Cheson 2014). * Pharmacokinetics (PK) parameters (e.g., AUC, Cmax) and incidence of antidrug antibodies (ADA).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous clinical and laboratory assessment from informed consent through 30-37 days post-treatment. **Serious Adverse Events (SAEs):** Reported promptly per global regulatory requirements and Sponsor pharmacovigilance protocols. **Data Monitoring Committee (DMC):** Internal safety review and dose-escalation committee to monitor DLTs and dictate transition from Part A to Part B/C cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety analysis set (all patients receiving ≥ 1 dose of SGN-35C); Efficacy/Response evaluable population. **Sample Size Justification:** Driven by a standard Phase 1 dose-escalation schema to identify the maximum tolerated dose (MTD) or recommended Phase 2 dose (RP2D), not powered for formal hypothesis testing. **Handling of Missing Data:** Standard missing data imputation rules applicable to Phase 1 oncology studies will be applied.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring by the Sponsor to ensure protocol compliance and data integrity.

Audit Trail: Electronic case report forms (eCRF) utilizing comprehensive audit trails and data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: {{SGN35C-001}} **Registry Number:** {{NCT06254495}}

Sponsor/Lead Organization: {{Seagen, a wholly owned subsidiary of Pfizer}}

Study Title: {{A Safety Study of PF-08046044/SGN-35C in Adults With Advanced Cancers}} **Official Regulatory Title:** {{A Phase 1, Open-label Study to Evaluate PF-08046044/SGN-35C in Adults With Advanced Malignancies}}

12. Clinical Framework (Lymphoma Specific)

Primary Condition: {{Relapsed/Refractory Lymphomas (cHL, PTCL, sALCL, DLBCL)}} **Diagnosis Threshold:** {{Histologically confirmed R/R disease; CD30 expression $\geq 1\%$ required for DLBCL/PTCL in dose-escalation}} **Medical Methodology:** {{CD30-directed antibody-drug conjugate targeting and cytotoxicity}} **Optimization Target:** {{Identification of optimal safety profile and RP2D for TOP1-inhibitor ADC}}

13. Study Procedures & Methodology

Management Pathway: {{First-in-human Dose Escalation (Part A), Dose Optimization (Part B), and Dose Expansion (Part C)}}

Education Protocol (HCP): {{Investigator training on DLT recognition and specialized AE management}}

Education Protocol (Patient): {{Informed consent and standard clinical trial participation orientation}}

Quality Audit Mechanism: {{Sponsor safety review committee and clinical data monitoring}}

14. Observation & Follow-Up Schedule

Total Duration: {{Treatment until progression or toxicity + 1 year safety/survival follow-up}} **Onsite Visit Cadence:** {{Every 3 weeks (Q3W) corresponding with treatment cycles}}

Remote Monitoring Frequency: {{Standard interval-based adverse event remote monitoring}}

Checklist Review Interval: {{Ongoing per cycle (Q3W) prior to each dose administration}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					